

A Postpartum Mind Journal

A 60-page noticing-and-tracking journal for the first year of new parenthood. Not a diagnosis tool. A way to see your own pattern — so that when you talk to someone who can help, you have something to show them.

Before you open this

If you are thinking about hurting yourself or your baby — right now, while reading this — please put this journal down and call **988** (US Suicide & Crisis Lifeline) or the **US Maternal Mental Health Hotline at 1-833-852-6262**. Both are 24/7, free, and confidential.

A journal is for noticing patterns. It is not for crisis. If you are in crisis, you need a human, not a page.

A note from me

I'm not a therapist. I'm not a doctor. I made this because — in the year after my second baby was born — I knew something was off and I could not have told you what.

I'd say "I'm fine" to the pediatrician and mean it. Then I'd cry in the parking lot and not know why. Then I'd be fine again by dinner. When I finally got to a therapist's office, she asked me how I'd been feeling and I genuinely could not remember. The past three months were a blur.

This journal is the thing I wish I'd had. Not to fix anything. Just to keep a record — so that when I sat down with a provider, I could hand them a few weeks of notes and say "here. This is what's been happening."

If you're using this, you might already suspect something is going on. Or someone close to you might. Or you might just be a person who wants to take care of yourself proactively in a season that is famously hard.

All of those are fine reasons.

— Soothemade

What this journal is, and is not

IS

- A daily mood + symptom log
- A weekly self-awareness inventory written in plain language
- CBT-style “notice the thought” prompts
- A list of signs that suggest it’s time to call a provider
- A page to prepare for a provider appointment
- A page your partner can use to share what they’ve been observing
- A tracking tool to take to a therapist or OB

IS NOT

- A diagnosis. We never use the words “you have” anything.
 - A replacement for therapy, medication, or a provider’s care.
 - A reproduction of any clinical screening tool. The weekly inventory in this journal uses our own plain language. If you want a clinically-validated screening, your OB or therapist will administer one (often the EPDS).
 - A guarantee. PPD and PPA are real medical conditions. A journal cannot treat them. A trained provider can.
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How to use it

- 60 pages, undated. Start whenever.
- Use it for one week at a time. Don't try to "catch up."
- Skip pages that don't apply. There is no rule.
- Bring it to your next appointment. You don't need to hand the whole thing over — flag a few pages. Even saying "I've been tracking this for three weeks" tells a provider a lot.

The names we use here

Different names get used for “postpartum mental health stuff.” Here’s what we mean when we say each:

- **Baby blues** — mood shifts, tearfulness, overwhelm in the first ~2 weeks postpartum. Affect ~80% of birthing parents. Usually resolve on their own by day 14.
- **Postpartum depression (PPD)** — persistent low mood, hopelessness, loss of interest, sleep/appetite changes, that don’t resolve. Can start any time in the first year. Affect ~1 in 7 birthing parents and ~1 in 10 non-birthing partners.
- **Postpartum anxiety (PPA)** — racing thoughts, intrusive scary thoughts, inability to relax even when baby is asleep, physical anxiety symptoms. Often overlaps with PPD but can occur alone. Estimated 1 in 5.
- **Postpartum OCD (PPOCD)** — intrusive, unwanted, often disturbing thoughts (often about harm to the baby), accompanied by checking or avoidance behaviors. The intrusive thoughts in PPOCD are not desires — they’re distressing precisely because the parent does NOT want them. Treatable. Important to flag to a provider.
- **Postpartum psychosis** — rare (~1–2 per 1000), but a medical emergency. Symptoms include: feeling disconnected from reality, hearing voices, paranoid thoughts, manic energy, beliefs that don’t fit reality. If this is you or someone you know — go to the ER. Now.

All of the above are treatable. None of them are character flaws. None of them mean you don’t love your baby.

Crisis lines — keep these handy

- **988** — US Suicide & Crisis Lifeline (24/7, call or text)
- **1-833-852-6262** — US Maternal Mental Health Hotline (24/7, free, confidential, parent-specific)
- **741741** — Crisis Text Line (text “HOME” to start)
- **911 / ER** — for postpartum psychosis symptoms or active self-harm thoughts

International readers: please look up your country’s crisis line and write it here: ____

DAILY MOOD LOG

Use one page per day. The format below repeats for ~30 days in the printed journal.

DAY ____ / DATE ____ / WEEKS POSTPARTUM ____

OVERALL MOOD TODAY (circle):

1 2 3 4 5 6 7 8 9 10
worst best

ANXIETY LEVEL TODAY (circle):

1 2 3 4 5 6 7 8 9 10
none constant

SLEEP LAST NIGHT:

Hours total: ____ Longest stretch: ____

Quality (circle): poor fair okay good

APPETITE TODAY (circle):

none ate-but-no-pleasure normal stress-eating

ENERGY TODAY (circle):

empty low okay normal wired-but-tired

CRYING TODAY (circle):

none once 2-3 times more than 3 constant

ONE THING THAT WENT BETTER THAN EXPECTED:

ONE THING THAT FELT HARDER THAN IT SHOULD HAVE:

A THOUGHT THAT KEPT COMING BACK TODAY (write it, don't judge it):

ANYTHING THAT SCARED ME ABOUT HOW I'M FEELING (write it; this page is for you and possibly a provider — no one else):

WEEKLY SELF-AWARENESS INVENTORY

Not a diagnosis. Not a clinical screening tool. A plain-language way to see your own pattern, week by week. Use one inventory at the end of each week.

WEEK OF: _____

In the past 7 days, how often did each of these happen?
(Use: never / rarely / some-days / most-days / every-day)

MOOD

I felt low, sad, or empty. _____

I cried more than I expected to. _____

I felt hopeless about the future. _____

I lost interest in things I usually enjoy. _____

I felt numb — like nothing was registering. _____

I felt guilty in a way that felt out of proportion. _____

I had thoughts that I'd be better off not here. _____

(If yes to this one — please call 988 or 1-833-852-6262.
You do not have to be in active danger to call. They can
help you think through next steps.)

ANXIETY

I felt anxious or "on edge" most of the day. _____

My heart raced or I felt physically panicky. _____

I worried about the baby in a way that felt
intrusive — I couldn't put it down. _____

I couldn't sleep even when the baby was asleep. _____

I had scary "what if" thoughts about something bad
happening to the baby. _____

I had thoughts I didn't want to have — that came in
from nowhere — and they scared me. _____

(If yes — this is something to bring to a provider.
Intrusive scary thoughts are common in postpartum
anxiety and OCD, and they are very treatable. Having
the thought does not mean you want to act on it.)

CONNECTION TO BABY

I felt connected to the baby this week. _____

I felt detached — like I was going through the
motions but not feeling it. _____

I felt resentment toward the baby that surprised me. _____

I struggled to feel "love" the way I thought I would. _____

(Important: NONE of these mean you are a bad parent.

Bonding is sometimes immediate. Often it isn't. Many parents — especially after hard births, NICU stays, or PPD — feel a delayed bond. This is treatable. Bring it up with a provider.)

CONNECTION TO YOURSELF

I felt like myself this week. _____

I felt like a version of me I don't recognize. _____

I had moments of joy. _____

I felt invisible — like no one was checking on me. _____

PHYSICAL

I slept whenever the baby slept. _____

I ate enough to fuel a body that's feeding / healing. _____

I drank enough water. _____

I left the house at least once. _____

I had pain or physical symptoms I'm ignoring. _____

THIS WEEK'S NOTES TO MY FUTURE SELF (OR A PROVIDER):

WHEN TO CALL YOUR PROVIDER — A SHORT LIST

Call sooner rather than later. You will not be “wasting their time.” Postpartum providers are trained to expect these calls.

Call within 24 hours if:

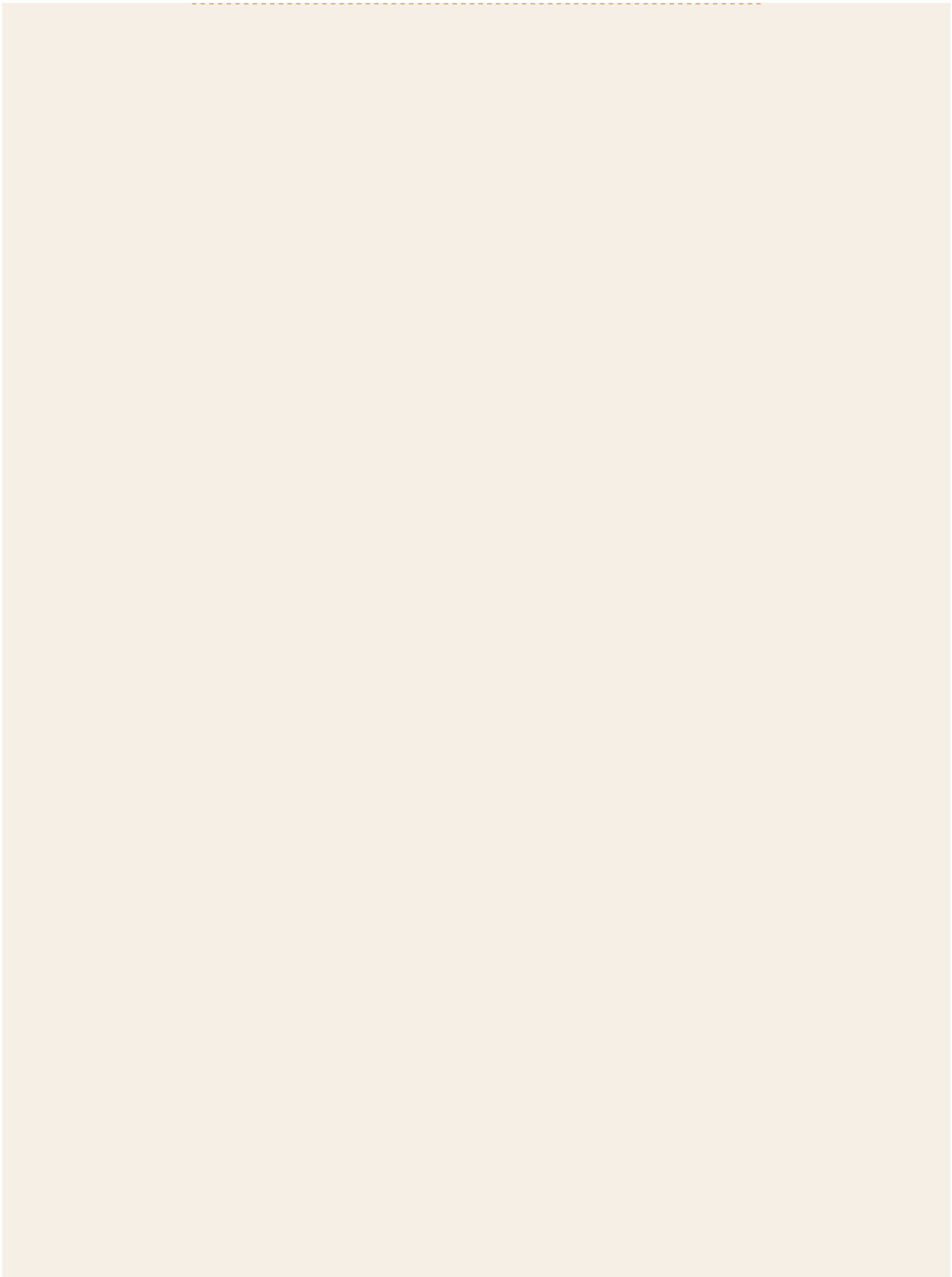
- You’ve felt low or hopeless for 2+ weeks straight
- Your anxiety is so high you can’t sleep when the baby sleeps
- You’re having intrusive scary thoughts — even occasionally
- You’re avoiding being alone with the baby
- You feel detached or “not yourself” most days
- You haven’t eaten or slept properly in 3+ days
- A friend or partner has gently asked if you’re okay and you brushed it off but you weren’t honest

Call today / go to the ER if:

- You have thoughts of hurting yourself or the baby
- You feel disconnected from reality, hearing or seeing things others don’t
- You have racing energy, can’t sleep at all, and feel “high” in a way that isn’t normal for you (possible postpartum psychosis — this is an emergency)
- You’ve stopped being able to care for the baby’s basic needs

It’s NOT “just baby blues” if:

- It’s been more than 2 weeks postpartum
- The low or anxious feelings are most days, not just bad moments
- You can’t picture yourself feeling better
- Sleeping more does not help
- It’s getting worse, not better



NOTICING THOUGHTS — CBT-STYLE PROMPTS

Twelve pages of these in the printed journal. One prompt per page. Use as many or as few as helps.

Page 1. A thought I had a lot this week:

What was happening when it came up?

Is this thought a fact, or a feeling? (Just notice — no need to judge.)

If a close friend told me she was having this thought, what would I say to her?

Page 2. A worry I keep returning to about the baby:

What is the most likely outcome, realistically?

What's the catastrophic outcome my brain keeps showing me?

What would help me move through the worry without solving it? (Sometimes worries don't get solved. They just get smaller as we move.)

Page 6. A thing my partner / family / friend said this week that landed wrong:

What would I have wanted them to say instead?

Is there a version of this I can ask for going forward?

Page 7. A thing I've been hiding from the people closest to me:

What am I afraid would happen if I said it out loud?

Who is the one person I could tell — even just half of it?

Page 8. What does "I'm fine" actually mean when I say it lately?

What's the more honest sentence underneath?

Page 9. An intrusive thought I've had this week that scared me:

(Important: writing this down does NOT make it more real. Intrusive thoughts are unwanted mental events. They are common in postpartum anxiety and OCD. Naming them helps reduce their power.)

How did I feel when the thought came? (afraid, ashamed, confused, all of the above?)

This is something to mention to a provider. They will not take the baby away for naming an intrusive thought. They will recognize it for what it is — a symptom — and help.

Page 10. When I imagine “feeling better,” what does that look like?

(Not “feeling great.” Just better than now.)

What’s one tiny step that might point toward that picture?

Page 11. Three things I’m afraid people are thinking about me as a parent:

1.

2.

3.

How likely is each of these to actually be true? What’s the evidence?

Page 12. A version of myself I used to be that I miss:

What did she have that I want back?

How much of that is fixable, and how much is just grief for a life-before-baby that I am allowed to miss?

APPOINTMENT PREP PAGE

Tear this out, or copy the answers. Bring it to your next OB / GP / therapist visit. This page makes the "How are you feeling?" question much easier to answer honestly.

DATE OF APPOINTMENT: _____ PROVIDER: _____

WEEKS POSTPARTUM: ____

I want to talk about (check all that apply):

- ☐ My mood has been low for ____ weeks
- ☐ My anxiety has been high for ____ weeks
- ☐ I'm having intrusive thoughts
- ☐ I'm having trouble sleeping when the baby sleeps
- ☐ I don't feel like myself
- ☐ I feel detached from the baby
- ☐ I've thought about not being here
- ☐ My partner / family has expressed concern
- ☐ I want to know about therapy options
- ☐ I want to know about medication options
- ☐ I want to ask about a referral to a perinatal mental health specialist (PMH-C credentialed providers exist)

WHAT I'VE BEEN TRACKING (point to the journal):

- Mood logs from: _____ to _____
- Weekly inventories: ____ completed
- Notes on intrusive thoughts: ☐ yes ☐ no
- Notes on sleep: ☐ yes ☐ no

WHAT I'M HOPING TO LEAVE THIS APPOINTMENT WITH:

- ☐ A plan (whether that's therapy, medication, both, or something else)
- ☐ A referral
- ☐ A follow-up appointment booked
- ☐ Permission to come back sooner if it gets worse
- ☐ Just to have said it out loud once

QUESTIONS I WANT TO ASK:

WHAT I MIGHT FORGET TO MENTION IF I DON'T WRITE IT DOWN:

A PAGE FOR YOUR PARTNER

If your partner is the one who first picked up this journal and is worried about you — this page is for them. If they're not, you can hand them this page when you're ready.

What you may have been noticing:

- More crying than usual, including over things that don't seem to warrant it
- Less interest in things she used to enjoy
- Withdrawal — from you, from friends, from the baby
- Anxiety that won't quiet, including at night
- Sleeping a lot, or not sleeping at all
- "I'm fine" said in a way that doesn't match the rest of the day
- Statements like "the baby would be better off without me"
- Anger that flashes hot and is gone
- Detachment from the baby in a way she's ashamed of

What helps:

- Notice without prescribing. "I've been noticing it's been a rough few weeks. How are you, really?" — not "You should call someone."
- Offer a specific next step. "I can make the call. I can drive you. I can stay with the baby. What do you want me to take off your plate?"
- Don't try to talk her out of how she feels. Listen first.
- Don't take her words as a personality test of you. Postpartum mental health is medical, not relational.

Concerning signs — if you see any of these, gently push for a same-week appointment:

- Statements about being a "bad mom," repeated
- "The baby would be better off without me"
- Talking about not being here
- Refusing food / not sleeping / not getting up

- Disconnected from reality

Emergency signs — call 988, or take her to the ER:

- Statements about harming herself or the baby
- Hearing or seeing things that aren't there
- Manic energy paired with no sleep + paranoid statements (possible postpartum psychosis)

Your role is not to fix this. Your role is to notice, to make space, and to help her get to the people whose job it is to help. You being steady is more valuable than you being right.

You may also be struggling. Non-birthing partners experience PPD at a rate of about 1 in 10. The same crisis lines apply to you.

NOTES — WHAT I'VE BEEN NOTICING (PARTNER):

Date: Observation:

_____	_____
_____	_____
_____	_____
_____	_____
_____	_____
_____	_____

MONTHLY REFLECTION

Once a month, fill this out. It's how you'll see whether things are trending in the direction you want.

MONTH OF: _____

WEEKS POSTPARTUM AT END OF MONTH: ____

LOOKING BACK AT THE WEEKLY INVENTORIES, WHICH PATTERN SHOWED UP MOST?

COMPARED TO LAST MONTH, MY OVERALL MOOD IS:

☐ Better ☐ About the same ☐ Worse ☐ Hard to say

COMPARED TO LAST MONTH, MY ANXIETY IS:

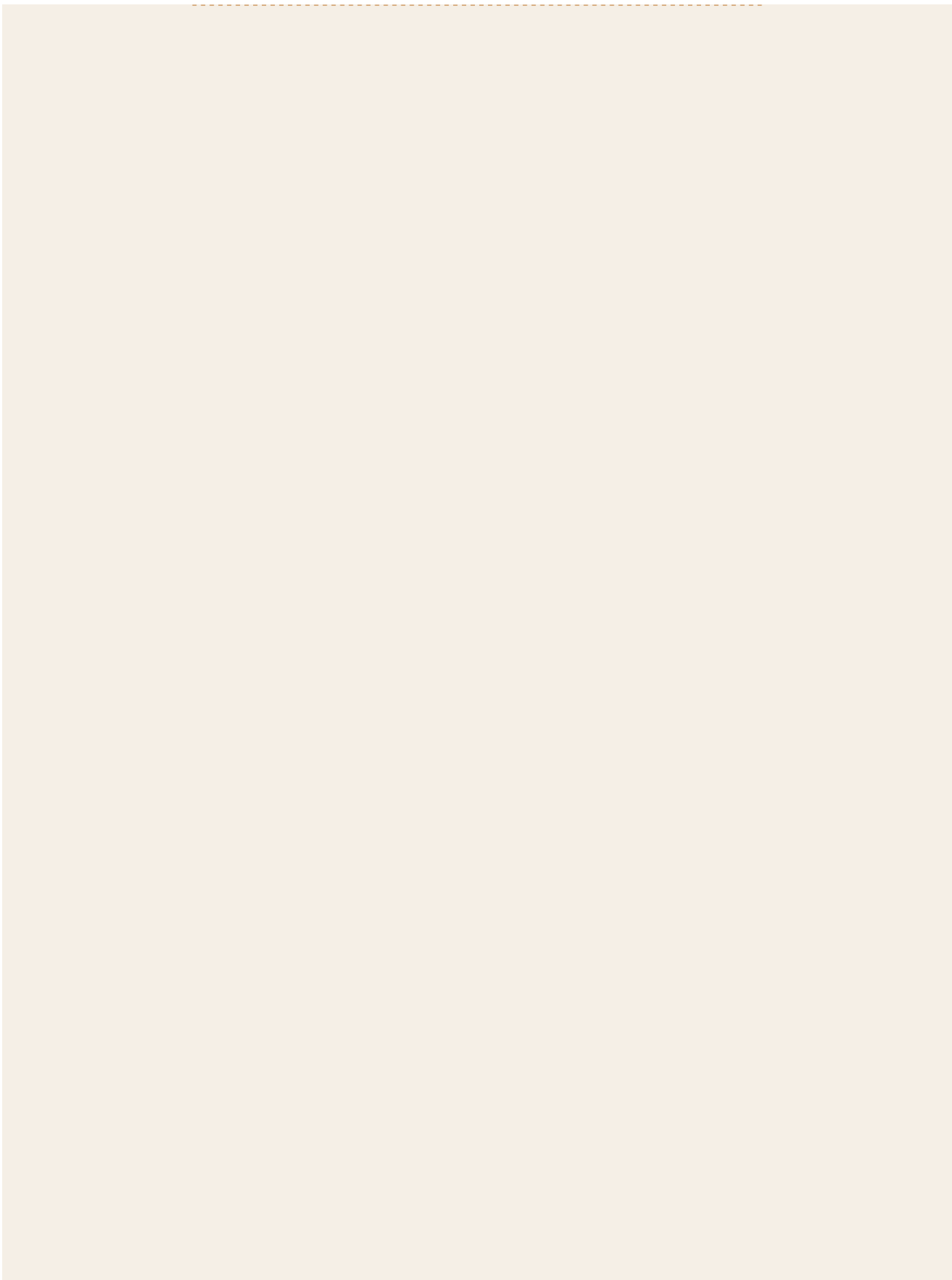
☐ Better ☐ About the same ☐ Worse ☐ Hard to say

WHAT I'M MOST PROUD OF FROM THIS MONTH:

WHAT I'D LIKE TO SHIFT NEXT MONTH (NOT "GOAL," JUST A DIRECTION):

A THING I'D TELL A FRIEND IN THE SAME SEASON:

ONE QUESTION FOR MY PROVIDER AT MY NEXT VISIT:



A LAST PAGE

You opened this journal. That is not a small thing.

Postpartum mental health is one of the most treatable areas of medicine, and one of the most under-treated — because so many parents do not ever name what they're feeling out loud. The cost of staying quiet is high.

You don't have to be in crisis to call. You don't have to be "sure" to ask. You don't have to wait for the 6-week appointment if it's getting worse before then.

If something in here gave you a sentence you can hand to a provider — that's enough. That was the point.

Whatever you're feeling — even if you can't name it yet — it is real. It is happening to your body and your brain at a chemically and structurally unique moment in your life. You did not cause it. You cannot will it away. And the people whose job it is to help are not going to be surprised by any of it.

The bravest sentence in postpartum motherhood is "I am not okay, and I think I need help." It is also the sentence that — over and over again — turns out to be the start of feeling better.

— Soothemade

RESOURCES — KEEP THIS PAGE

- **US Maternal Mental Health Hotline:** 1-833-852-6262 (24/7, free, confidential, English/Spanish + interpreters)
- **988 Suicide & Crisis Lifeline:** call or text 988
- **Crisis Text Line:** text HOME to 741741
- **Postpartum Support International:** postpartum.net — find a perinatal mental-health-certified provider (PMH-C); has support groups, including for partners and dads
- **2020 Mom:** 2020mom.org — advocacy and resources
- **The Bloom Foundation / Postpartum Health Alliance** (regional): search “perinatal mental health [your state]”

This journal is a noticing and tracking tool. It is not medical advice, diagnostic guidance, or a replacement for professional mental-health care. The inventories in this journal use plain language and are not validated clinical screening instruments — if you want a clinically-validated screening (such as the EPDS), ask your provider. If you are experiencing a mental-health emergency, contact a provider, go to the ER, call 988, or call the US Maternal Mental Health Hotline at 1-833-852-6262.